



Care Indeed Home Health

Mock Patient Evaluation Document · Version 1.0 MOCK · Training Date
July 1, 2026

MOCK TRAINING RECORD
SYNTHETIC UAT DATA
NOT FOR CLINICAL USE
DO NOT UPLOAD TO LIVE PATIENT
CHART

EVALUATION 7 — INCOMPREHENSIBLY COMPLEX ("BREAK THE ADMINISTRATOR")

Primary Diagnosis: Pediatric-to-Adult Transition with Congenital Heart Disease (Single Ventricle Physiology, Status Post Fontan), Heart-Lung Transplant Evaluation, Home Inotrope Infusion, BiPAP, Jejunostomy Feeds, Anticoagulation, Protein-Losing Enteropathy, Plastic Bronchitis, Liver Cirrhosis (Fontan-Associated), Renal Failure on Peritoneal Dialysis, Sickle Cell Trait Interaction, Severe Developmental Delay, Non-Verbal with Augmentative Communication, Legal Conservatorship, and Multi-Agency Care Coordination

PATIENT DEMOGRAPHICS

Field	Detail
Patient Name	Esperanza "Espie" Milagros Reyes
Date of Birth	09/17/2003
Age	21
Sex	Female
Race/Ethnicity	Hispanic/Latina (Filipino and Mexican American, multiethnic)
Primary Language	Non-verbal — uses AAC device (Tobii Dynavox Snap + Core First); understands English and Tagalog
Interpreter Needed	Yes — Tagalog interpreter required when communicating with mother (primary language Tagalog; understands conversational English but not complex medical terminology)
Marital Status	Single
Medical Record Number	CI-2025-00463
Address of Service	7540 Bayshore Circle, Apt 3A (ground floor, ADA-adapted), Pacifica, CA 94044
Cell Phone	N/A (patient non-verbal; mother cell below)
Mother Cell	(650) 555-0327
Email	maria.reyes.care@email.com (mother's email; monitors on patient's behalf)
County	San Mateo

LEGAL STATUS

Field	Detail
Conservator	Maria Santos Reyes (mother) — Limited conservatorship granted 09/2021 by San Mateo County Superior Court (Case # _____). Conservatorship covers: medical decision-making, financial decisions, residence decisions, and education decisions.
Conservatorship Documentation	Letters of Conservatorship on file (copy in clinical record).
Capacity	Patient does not have decision-making capacity for complex medical decisions due to moderate-severe intellectual disability (IQ estimated 35-40 per neuropsychological evaluation 2022). Patient can express basic preferences (food, comfort, like/dislike, pain) via AAC device and facial expression. Patient demonstrates understanding of simple explanations but cannot comprehend complex risk/benefit analyses or provide informed consent. All consents signed by conservator (mother).
Regional Center	Golden Gate Regional Center — Service Coordinator: Sandra Iglesias, (650) 555-0991
IHSS	Active — 283 hours/month authorized (mother is paid IHSS provider)
SSI/SSDI	SSI recipient — Representative Payee: Maria Santos Reyes

EMERGENCY CONTACTS

Priority	Name	Relationship	Phone	Lives with Patient?
1	Maria Santos Reyes	Mother / Conservator	(650) 555-0327 (cell) / (650) 555-0328 (home)	Yes
2	Ricardo Reyes	Father	(650) 555-0412	Yes (parents married, both in home)
3	Isabella Reyes	Sister (age 17 — trained caregiver, high school senior)	(650) 555-0329	Yes
4	Lucene Heart-Lung Transplant Coordinator	Stanford	(650) 555-1675 (24-hour transplant line)	N/A
5	Filipino Community Health Worker	Gawad Kalinga Clinic	(650) 555-0850	N/A — community navigator for family

Other persons in home: Grandmother (Lola Consuelo, age 78, dementia — patient's grandmother requires some supervision herself; lives in home; complicates caregiver burden)

LIVING SITUATION

Field	Detail
Lives With	Mother Maria (52), Father Ricardo (55, works construction — away 10-12 hrs/day), Sister Isabella (17 — helps after school/weekends), Grandmother Consuelo (78, mild-moderate dementia — requires supervision)
Residence	3BR ground-floor ADA-adapted apartment; wheelchair accessible; roll-in shower; widened doorways
DME	(see extensive DME section below)
Pets	1 small dog ("Bituin," Pomeranian, 6 lbs) — emotional support animal; patient's primary comfort; non-allergenic food only near patient
Cultural/Religious	Devout Catholic; faith is central to family coping. Weekly communion brought to home by parish eucharistic minister. Cultural integration of Filipino healing practices (hilot — traditional massage; mother applies to patient's extremities; not harmful, may be beneficial for comfort/circulation). Holy water and religious items at bedside per family custom — accommodate.

DME INVENTORY

Equipment	Details
BiPAP	ResMed AirCurve 10 VAuto; settings: IPAP 18, EPAP 8, Rate 14; used during sleep (10 PM – 7 AM) and during daytime naps; patient NOT vent-dependent but requires NIV for sleep and periods of fatigue/decompensation
Pulse Oximeter	Continuous overnight; spot-check Q4H daytime; alarm set at <88%
Suction Machine	Portable DeVilbiss; for oral suctioning (excessive secretions — plastic bronchitis)
Cough Assist	Philips CoughAssist E70; settings: +40/-40 cmH2O; used BID + PRN for cast/plug expectoration (plastic bronchitis)
Nebulizer	PARI LC Plus; for inhaled medications
O2 Concentrator	5 LPM concentrator; PRN for desaturation (SpO2 <88%); patient not on continuous O2 at baseline
Infusion Pump (Milrinone)	CADD-Solis ambulatory pump; milrinone infusion via Hickman catheter 24/7; THIS IS A CONTINUOUS, LIFE-SUSTAINING INFUSION — abrupt discontinuation can cause acute decompensation and death
Peritoneal Dialysis Cycler	Baxter HomeChoice Claria; nightly automated PD (10 PM – 7 AM); dwells programmed per nephrology
J-Tube Feeding Pump	Kangaroo ePump; continuous feeds 18 hrs/day
Hospital Bed	Full-electric with alternating pressure mattress
Wheelchair	Permobil M3 Corpus tilt-in-space; head-array drive control (patient cannot use hands for joystick); custom seating
Hoyer Lift	Portable (ceiling track not feasible in apartment)
Shower/Commode Chair	Tilt-in-space shower chair
AAC Device	Tobii Dynavox I-Series with Snap + Core First; eye-gaze access; mounting arm on wheelchair; secondary device: iPad with TouchChat
Cardiac Monitor	Portable telemetry (event monitor) — for arrhythmia detection; transmitted daily to cardiology
Specialty Supplies	PD supplies (delivered monthly — Baxter), Hickman catheter supplies, J-tube supplies, wound care supplies, cast expulsion supplies (specimen cups for bronchial casts), emergency medication kit

PAYER INFORMATION

Priority	Payer	ID / Policy Number
Primary	Medi-Cal Managed Care (Health Plan of San Mateo — CCS transition)	HPSM-90518833E
Secondary	California Children's Services (CCS) — transitioning to CCS Whole Child Model via HPSM	CCS-SM-2025-07741
Tertiary	SSI/Medi-Cal	Linked

Note: This patient is transitioning from pediatric to adult services. CCS has historically covered all cardiac-related care. Under the CCS Whole Child Model, HPSM now coordinates CCS and non-CCS services through a single managed care plan. Prior authorizations are complex — some services route through CCS carve-out, others through HPSM directly. Agency billing team must coordinate closely with HPSM CCS care coordinator for authorization of all services.

CCS Care Coordinator: Jennifer Lam, (650) 555-0744 HPSM Care Manager: Patricia Okonkwo, RN, (650) 555-0288

PHYSICIAN INFORMATION (9 specialists + PCP)

Role	Name	Specialty	Phone
Attending / Certifying	Dr. Michael Chen	Adult Congenital Heart Disease (ACHD)	(650) 555-1700
Heart-Lung Transplant	Dr. Elizabeth Hartfield	Cardiothoracic Surgery / Transplant	(650) 555-1675
Pulmonologist	Dr. Sarah Atkins	Pulmonary	(650) 555-1135
Nephrologist	Dr. Amir Hussain	Nephrology (Peritoneal Dialysis)	(650) 555-1920
Hepatologist	Dr. Michelle Park	Hepatology	(650) 555-1605
GI	Dr. Heather Kim	Gastroenterology	(650) 555-1320
Hematologist	Dr. Laverne Okafor	Hematology (anticoagulation + sickle trait)	(650) 555-1830
Developmental Medicine	Dr. Priya Mehta	Developmental-Behavioral Pediatrics (transitioning to adult)	(650) 555-1122
Psychiatrist	Dr. Nina Kovacs	Psychiatry	(650) 555-1780
PCP	Dr. Evelyn Nakamura	Family Medicine	(650) 555-0400

F2F Encounter Date: 06/06/2025 (ACHD clinic visit) F2F Performed By: Dr. Michael Chen

REFERRAL / HOSPITALIZATION HISTORY (extensive — lifelong cardiac patient)

Event	Facility	Dates	Reason
Most Recent Admission	Lucile Packard / Stanford	05/10/2025 – 06/08/2025	Acute decompensated heart failure (Fontan failure); started milrinone infusion; Hickman placed for home inotrope; initiated heart-lung transplant evaluation; plastic bronchitis exacerbation requiring bronchoscopy x2; PD catheter placed (Fontan-associated renal failure)
Previous Admission	Lucile Packard	02/2025	Plastic bronchitis — respiratory distress, bronchoscopy, ICU admission
Previous Admission	Lucile Packard	10/2024	PLE flare — severe hypoalbuminemia (albumin 1.2), massive edema, IV albumin replacement
Previous Admission	Lucile Packard	06/2024	Atrial flutter with rapid ventricular response; cardioversion; amiodarone started

Surgical / Procedural History (cardiac — lifelong):

Neonatal: Norwood Stage I (2003)

Infancy: Bidirectional Glenn (2004)

Age 3: Fontan completion — extracardiac conduit (2006)

Age 12: Fontan revision with fenestration (2015)

Age 16: PLE diagnosed; initiated budesonide and heparin protocol (2019)

Age 18: Pacemaker implantation — epicardial, dual-chamber (2021) for sick sinus syndrome

Age 21: Hickman catheter placement, PD catheter placement (2025)

Multiple cardiac catheterizations (7 total through 2025)

Multiple bronchoscopies (4 total — plastic bronchitis cast removal)

Jejunostomy placement (2022) for enteral nutrition (oral feeding insufficient)

DIAGNOSES (38+ active diagnoses — most complex patient in this set)

Priority	ICD-10	Diagnosis
Primary	Q23.4	Hypoplastic left heart syndrome (single ventricle physiology)
Active	Z95.812	Status post Fontan procedure (extracardiac conduit, 2006; revised with fenestration, 2015)
Active	I50.814	Right heart failure (Fontan failure — decompensated; on home milrinone)
Active	Z94.89	Status of heart-lung transplant evaluation (listed/evaluating — UNOS status per transplant team)
Active	Z45.09	Encounter for adjustment of cardiac device — pacemaker (epicardial, dual-chamber; battery check Q6 months)
Active	I48.3	Typical atrial flutter (history — cardioverted; on amiodarone)
Active	I49.5	Sick sinus syndrome (pacemaker-dependent — paced rhythm)
Active	Z95.0	Presence of cardiac pacemaker
Active	K90.89	Other intestinal malabsorption — protein-losing enteropathy (PLE) — Fontan-associated
Active	E44.0	Moderate protein-calorie malnutrition (albumin 2.0; prealbumin 8)
Active	J98.09	Other diseases of bronchus, NEC — plastic bronchitis (Fontan-associated; recurrent cast formation; requires CoughAssist + PRN bronchoscopy)
Active	J96.11	Chronic respiratory failure with hypoxia (baseline SpO2 88-92% on RA)
Active	K76.7	Hepatorenal syndrome (may be more accurately coded as Fontan-associated liver disease)
Active	K74.69	Other cirrhosis of liver — Fontan-associated liver disease (FALD); congestive hepatopathy; MELD-XI 18; portal hypertension; esophageal varices (Grade 1, not yet banded)
Active	N18.5	Chronic kidney disease, stage 5 (GFR 12 — on peritoneal dialysis)
Active	Z99.2	Dependence on renal dialysis (automated peritoneal dialysis, nightly)
Active	Z99.81	Dependence on supplemental oxygen (PRN — not continuous)
Active	Z93.4	Jejunostomy status (J-tube — primary nutrition route)
Active	D57.3	Sickle cell trait (Hb AS) — clinically relevant due to chronic hypoxemia; risk of sickling events at SpO2 <85%; maintain SpO2 ≥88% at all times
Active	Z79.01	Long-term anticoagulant use (warfarin — Fontan circuit; enoxaparin bridge PRN)
Active	I82.891	Acute embolism and thrombosis — Fontan circuit thrombosis history (2020); on warfarin
Active	E87.1	Hyponatremia (chronic — dilutional; Na 129)
Active	E87.6	Hypokalemia (PD-related losses)
Active	E83.42	Hypomagnesemia
Active	E55.9	Vitamin D deficiency
Active	D63.1	Anemia in chronic kidney disease (Hgb 8.8; on darbepoetin)
Active	F72	Severe intellectual disability (IQ estimated 35-40; non-verbal; requires full assistance with all ADLs)

Active	F84.0	Autism spectrum disorder (diagnosed age 5; non-verbal; stereotypic behaviors — hand-flapping, rocking, which increase when distressed or in pain)
Active	F31.32	Bipolar II disorder, current episode depressed, moderate (diagnosed 2023; manifests as periods of agitation, self-injurious behavior — head-hitting, skin-picking — alternating with withdrawal/anhedonia)
Active	R45.89	Other symptoms and signs involving emotional state — episodic agitation and self-injurious behavior
Active	G40.909	Epilepsy, unspecified, not intractable (seizure history — last seizure 2024; on levetiracetam)
Active	L89.152	Pressure ulcer, sacral region, Stage 2 (2.0 cm x 1.5 cm, granulating)
Active	L89.019	Pressure ulcer, right elbow, unstageable (eschar — dry, stable, intact; do not debride per wound care MD — monitoring only)
Active	R62.52	Short stature (due to chronic illness; 4'9", 78 lbs)
Active	E46	Unspecified protein-calorie malnutrition
Active	R63.6	Underweight (BMI 13.1 — severely underweight)
Active	K22.10	Esophageal varices without bleeding (Grade 1 — Fontan portal HTN)
Active	R00.1	Bradycardia (pacemaker-managed)

ALLERGIES

Allergen	Reaction	Severity
Heparin	Heparin-induced thrombocytopenia (HIT) Type II — confirmed 2020	LIFE-THREATENING — NO HEPARIN IN ANY FORM (including flushes, coated catheters, line priming). Use NS flushes only. Argatroban for anticoagulation bridge if IV needed.
Morphine	Respiratory depression (near-intubation event, 2019)	SEVERE
Amoxicillin	Serum sickness–like reaction (fever, rash, joint pain)	Severe
Latex	Anaphylaxis	LIFE-THREATENING
Iodine contrast	Severe urticaria + bronchospasm (required epinephrine 2021)	Severe
Eggs	Angioedema	Moderate — relevant to enteral formula selection
Soy	GI intolerance (cramping, diarrhea)	Moderate — relevant to enteral formula selection

Formula note: Patient requires egg-free, soy-free enteral formula. Current: Peptamen 1.5 (Nestle) — semi-elemental, egg-free, soy-free; tolerated. Confirmed with GI/RD.

CURRENT MEDICATIONS (42 medications — maximum complexity)

#	Medication	Dose	Route	Frequency	Purpose	Critical Notes
1	Milrinone	0.375 mcg/kg/min	IV continuous via Hickman	24/7	Inotropic support (Fontan failure)	LIFE-SUSTAINING. DO NOT DISCONTINUE. Abrupt cessation → acute hemodynamic collapse. CADD pump; pharmacy-prepared bags delivered weekly. Prime/load/rate change per specific protocol.
2	Warfarin	3 mg	PO (via J-tube, crushed, suspended in water)	Daily (5 PM)	Anticoagulation (Fontan circuit)	INR target 2.5-3.5 (higher target due to Fontan). NO HEPARIN — HIT history. NS flushes only. Draw INR 2x/week.
3	Amiodarone	200 mg	PO (via J-tube)	Daily	Atrial flutter prophylaxis	Monitor thyroid (TSH Q3 months), LFTs (already abnormal — Fontan liver; coordinate with hepatology), pulmonary (PFTs if feasible), corneal (ophthalmology annual). Interacts with warfarin (increases INR — dose stabilized but monitor closely).
4	Budesonide (Entocort)	9 mg	PO (via J-tube — do NOT crush capsule contents further; open capsule and pour granules into J-tube with water flush)	Daily AM	Protein-losing enteropathy (PLE)	Part of PLE management triad (budesonide + albumin + heparin-alternative); monitor for adrenal suppression, infection risk.

5	Spirolonolactone	25 mg	PO (via J-tube)	BID	Diuresis; Fontan failure; ascites; hepatic	Monitor K+ (risk of hyperkalemia with CKD5 + spironolactone + PD); hold if K+ >5.5
6	Furosemide	40 mg	PO (via J-tube)	BID	Diuresis; volume overload	Monitor electrolytes; adjust with PD adequacy
7	Sildenafil (Revatio)	20 mg	PO (via J-tube)	TID	Pulmonary vascular resistance reduction (Fontan optimization)	Monitor BP; avoid nitrates.
8	Bosentan (Tracleer)	62.5 mg	PO (via J-tube)	BID	Pulmonary vasodilator (Fontan optimization)	Monthly LFTs required (hepatotoxicity — already compromised liver); pregnancy prevention required (teratogenic — patient has conservatorship, verify contraception is addressed).
9	Levetiracetam (Keppra)	500 mg	PO (via J-tube)	BID	Seizure prophylaxis	Monitor for behavioral side effects (may worsen agitation in ASD/ID population).
10	Risperidone	1 mg	PO (via J-tube)	BID	Agitation, SIB (self-injurious behavior), bipolar	Monitor for metabolic effects (weight, glucose); QTc monitoring (already on amiodarone — additive QT prolongation concern); tardive dyskinesia screening Q6 months.
11	Sertraline	75 mg	PO (via J-tube)	Daily AM	Depression / anxiety / ASD-related irritability	Drug interaction with amiodarone (QT).

12	Darbepoetin alfa (Aranesp)	40 mcg	SubQ	Every 2 weeks	Anemia (CKD)	Target Hgb 10-11; monitor iron studies.
13	IV Albumin 25%	50 mL (12.5 g)	IV via Hickman	2x/week (Mon, Thu)	PLE — albumin replacement	Infuse over 2 hours. Monitor for fluid overload (BiPAP + PD night may mitigate).
14	Peritoneal Dialysis	Per nephrologist prescription (4 exchanges, 1.5% dextrose, 2000 mL dwell volume)	PD catheter	Nightly (automated cycler 10 PM – 7 AM)	ESRD / CKD5	PD and BiPAP run simultaneously overnight. Train caregivers on setup/disconnect for both. Monitor exit site for peritonitis signs. PD effluent: clear/straw — report if cloudy (peritonitis).
15	Enteral formula (Peptamen 1.5)	50 mL/hr	J-tube (continuous pump)	18 hrs/day (6 AM – 12 AM)	Primary nutrition	Egg-free, soy-free, semi-elemental. 1350 kcal/day + modular protein. Free water flushes 150 mL Q4H.
16	Modular protein powder	6 g	Via J-tube (mixed in water flush)	QID	Additional protein (PLE losses)	
17	Pancrelipase (Creon)	24,000 units	PO/via J-tube	With formula initiation and Q6H	Fat malabsorption (PLE)	Swallow capsule contents — do not crush microspheres
18	Ursodiol	300 mg	PO (via J-tube)	TID	Fontan-associated liver disease	
19	Lactulose	15 mL	PO (via J-tube)	BID (titrate to 2-3 soft BMs/day)	Hepatic encephalopathy prophylaxis (Fontan liver)	
20	Rifaximin (Xifaxan)	550 mg	PO (via J-tube)	BID	Hepatic encephalopathy prophylaxis (adjunct)	Monitor LFTs
21	Albuterol	2.5 mg/3 mL	INH (nebulizer)	Q4H + PRN	Bronchospasm, secretion mobilization (plastic bronchitis)	

22	Hypertonic Saline 3%	4 mL	INH (nebulizer)	BID	Mucolytic — plastic bronchitis cast prevention/mobilization	Monitor SpO2 during; may trigger bronchospasm — give albuterol first
23	Dornase alfa (Pulmozyme)	2.5 mg	INH (nebulizer)	Daily	Mucolytic — plastic bronchitis	
24	N-Acetylcysteine (Mucomyst)	10% 3 mL	INH (nebulizer)	BID	Mucolytic — plastic bronchitis (adjunct)	Bronchospasm risk — give after albuterol
25	Pantoprazole	40 mg	PO (via J-tube)	Daily	GI prophylaxis; variceal protection	
26	Sucralfate	1 g	PO (via J-tube)	QID (1 hr before meals/feeds — schedule around J-tube feeds)	GI mucosal protection (varices)	Interacts with many meds — give 2 hrs apart from warfarin, levothyroxine, etc.
27	Calcium carbonate	500 mg	PO (via J-tube)	TID with feeds	Phosphorus binder (CKD) + calcium supplement	
28	Cholecalciferol (Vit D3)	50,000 IU	PO (via J-tube)	Weekly	Vitamin D deficiency (CKD + malabsorption)	
29	Folic acid	1 mg	PO (via J-tube)	Daily	Anemia support	
30	Ferrous sulfate (liquid)	15 mg elemental Fe	PO (via J-tube)	Daily (give 2 hrs apart from PPI, calcium, and sucralfate)	Iron deficiency	
31	Magnesium oxide	400 mg	PO (via J-tube)	BID	Hypomagnesemia	
32	Potassium chloride (liquid)	20 mEq	PO (via J-tube)	Daily (adjust per labs)	Hypokalemia	Careful — spironolactone + PD may swing K+ both ways
33	Sodium chloride tabs	1 g	PO (via J-tube)	TID	Hyponatremia	
34	Zinc sulfate	220 mg	PO (via J-tube)	Daily	Wound healing + PLE losses	
35	Vitamin A	10,000 IU	PO (via J-tube)	Daily	Fat-soluble vitamin deficiency (malabsorption)	Monitor levels — hepatotoxicity risk in liver disease
36	Vitamin E	400 IU	PO (via J-tube)	Daily	Fat-soluble vitamin deficiency	

37	Vitamin K1 (phytonadione)	5 mg	PO (via J-tube)	Weekly (separate day from warfarin dose change)	Fat-soluble vitamin deficiency + liver coagulopathy BUT on warfarin — dose carefully balanced	Vitamin K antagonizes warfarin; dose is precisely balanced by hematology — do NOT adjust without MD order
38	Multivitamin (liquid, pediatric formulation)	5 mL	PO (via J-tube)	Daily	General	Pediatric formulation due to patient size and J-tube tolerance
39	Melatonin	3 mg	PO (via J-tube)	HS	Sleep regulation (ASD)	
40	Acetaminophen (liquid)	325 mg	PO (via J-tube)	Q6H PRN	Pain / fever	Reduced dose — liver disease. Max 2g/day. NO NSAIDs (varices, CKD, anticoagulation).
41	Chlorhexidine oral rinse 0.12%	15 mL swish-and-spit	Oral	BID	Oral hygiene (immunocompromised, on inotropes — endocarditis prophylaxis adjunct)	Mother assists; patient cannot swish and spit independently — apply with oral swabs
42	Emergency medications	Epinephrine auto-injector (EpiPen Jr) — for anaphylaxis (multiple severe allergies); Diazepam rectal gel (Diastat) 10 mg — for seizure >5 min	IM / PR	PRN EMERGENCY	Anaphylaxis; status epilepticus	Must be immediately accessible at all times. All caregivers trained.

TOTAL MEDICATIONS: 42 IV Medications: 2 (milrinone continuous, albumin 2x/week) Controlled Substances: Diazepam rectal gel (emergency only) Hickman Catheter: Dual-lumen; Lumen 1: milrinone continuous; Lumen 2: albumin infusions, labs. NO HEPARIN FLUSHES — HIT HISTORY. NS FLUSHES ONLY. PD Catheter: Tenckhoff, left lower abdomen. Exit site care per nephrology protocol. J-Tube: Mic-Key low-profile 14 Fr jejunostomy.

CLINICAL ASSESSMENT — VITAL SIGNS AT ADMISSION

Parameter	Value	Notes
BP	88/52	Baseline for Fontan patient on milrinone — hypotension is expected. Contact ACHD if SBP <80 or symptomatic.
HR	72 (paced rhythm — VVIR)	Pacemaker-dependent
RR	24	Mildly elevated — baseline for this patient (restrictive lung disease)
Temp	98.6°F	WNL
SpO2	89% on RA	Baseline for Fontan patient (cyanotic at baseline). Alarm at <85%. Sickle cell trait — maintain ≥88% to avoid sickling.
Weight	78 lbs (35.4 kg)	Severely underweight; BMI 13.1
Height	4'9" (144.8 cm)	Short stature (chronic illness)
Pain	Unable to self-report; FLACC Scale: 4/10 (occasional grimacing, restlessness, intermittent guarding of abdomen)	Use FLACC or r-FLACC (revised for cognitively impaired) at each visit
Blood Glucose	92 mg/dL	WNL

HOMEBOUND STATUS

CONFIRMED — profoundly homebound. Non-ambulatory (wheelchair-dependent, head-array control). Connected to milrinone infusion pump 24/7. Connected to PD cyclor 10 hrs/day. Connected to J-tube pump 18 hrs/day. Requires BiPAP for sleep. Non-verbal with severe intellectual disability — unable to manage self in any community setting. Leaving home requires wheelchair-accessible transport, portable vent/BiPAP, infusion pump, suction, pulse oximeter, emergency medication kit, and a trained attendant capable of managing all devices and responding to emergencies. Taxing effort is extreme. Absences limited to Stanford ACHD clinic visits (monthly) and emergency department visits.

ADVANCE DIRECTIVES

Document	Status
AHCD	Executed by conservator (mother Maria) — 06/2025. Maria designated as sole decision-maker. Specific instructions: Full aggressive treatment including intubation, mechanical ventilation, CPR, transplant if offered. Maria states: "She is a fighter. She has survived everything. God will decide when it's her time, not us."
POLST	Full treatment (all sections).
Transplant Status	Patient is being evaluated for heart-lung transplant at Stanford. If listed and organ offer received, agency will be notified by transplant coordinator; patient will be transported emergently to Stanford for transplant surgery.

SERVICES ORDERED

Service	Discipline	Frequency	Duration
Skilled Nursing	RN	Daily x 4 weeks (IV milrinone management, albumin infusions, Hickman care, PD oversight, J-tube management, wound care, labs, teaching, respiratory treatments); then 5x/week ongoing	Indefinite
Physical Therapy	PT	3x/week	Indefinite
Occupational Therapy	OT	3x/week	Indefinite
Speech-Language Pathology	SLP	2x/week (AAC optimization, oral-motor, swallow safety)	Indefinite
Medical Social Work	MSW	1x/week	Indefinite
Home Health Aide	HHA	BID (AM + PM — personal care, repositioning, equipment assist)	Indefinite
Respiratory Therapy	RT (if available; otherwise RN + DME RT)	2x/week (CoughAssist, nebulizer protocol, BiPAP compliance)	Indefinite
Registered Dietitian	RD	Biweekly (J-tube formula optimization, PLE nutritional management)	Indefinite

SKILLED NURSING FOCUS AREAS — SUMMARY (full detail would require its own sub-document)

Milrinone Infusion Management — Verify pump rate, bag volume, line integrity, Hickman patency EVERY visit. Teach caregiver bag changes. Emergency: if pump fails → do NOT abruptly stop → manually maintain drip if possible → call transplant coordinator + 911 immediately. Cardiac monitor data transmission daily.

Hickman Catheter Care — Exit site assessment, dressing change weekly (NO heparin — NS flush only), blood draws via line (Lumen 2, per protocol), CLABSI prevention bundle.

PD Management — Assess PD catheter exit site (peritonitis signs: cloudy effluent, abdominal pain, fever). Record PD volumes (infused vs. drained = ultrafiltration). Coordinate with nephrology for prescription adjustments. Teach caregiver troubleshooting (alarms, drainage failure, leak).

Plastic Bronchitis Management — Aggressive pulmonary toilet: nebulizer schedule (albuterol → hypertonic saline → dornase alfa → Mucomyst → NS; spaced Q2-4H), CoughAssist BID + PRN, suctioning PRN. Monitor for cast expectoration — patient may cough up white/grey rubbery bronchial casts. Save casts in specimen cups for MD review. If cast obstructs airway → suction → CoughAssist → if persistent respiratory distress → call 911 (may need emergent bronchoscopy).

Fontan Failure / Cardiac Monitoring — Daily weight, I&O, BP, HR, SpO₂, edema assessment, ascites assessment (abdominal girth), JVD, hepatjugular reflux, peripheral perfusion. Report: weight gain >1 lb/day, new/worsening edema, SpO₂ <85%, SBP <80, new arrhythmia symptoms.

PLE Management — Albumin infusion 2x/week per protocol. Monitor albumin levels weekly (target >2.5). Assess edema. J-tube protein supplementation compliance. Coordinate with GI/ACHD.

Liver Disease Monitoring — LFTs per schedule. Assess for hepatic encephalopathy (confusion beyond baseline, sleep-wake reversal, asterixis — limited assessment in non-verbal patient; rely on behavioral changes reported by mother). Lactulose/rifaximin compliance. Monitor for variceal bleeding (hematemesis, melena, hematochezia — report immediately).

Anticoagulation — Warfarin INR 2x/week (target 2.5-3.5). Extremely complex drug interaction profile — amiodarone, budesonide, ursodiol, sucralfate, Vitamin K, enteral feeds ALL affect INR. Any medication change → anticipate INR shift → increase monitoring. NO HEPARIN — HIT.

Sickle Cell Trait Monitoring — Maintain SpO₂ ≥88%. If SpO₂ drops <85% despite O₂ supplementation → call 911 (sickling crisis risk). Ensure adequate hydration. Avoid extreme temperature exposure.

Seizure Management — Levetiracetam compliance. Seizure precautions: padded bed rails, nothing in mouth during seizure, time seizure, Diastat if >5 min, call 911. Document and report all seizure activity.

Wound Care — Sacral Stage 2: wound care per orders. Right elbow unstageable (eschar): monitor only, do not debride per wound care MD. Comprehensive skin assessment every visit. Braden 10 (very high risk). Repositioning Q2H.

Behavioral/Psychiatric — Monitor for SIB (self-injurious behavior), agitation, withdrawal. Use behavioral tracking log. Coordinate with psychiatrist. Risperidone monitoring (QTc, metabolic, tardive dyskinesia). Environmental modifications for sensory needs (ASD — low lighting, low noise, weighted blanket, comfort items, dog "Bituin" at bedside).

Multi-Agency Coordination — This patient has: home health (CI), IHSS, Regional Center, CCS, HPSM care manager, transplant team, PD supply vendor (Baxter), infusion pharmacy (Coram), DME vendor (multiple), and 10 physicians. RN serves as central care coordinator. Maintain communication log. Monthly care conferences (virtual) with all providers recommended.

Caregiver Support — Mother Maria is carrying an extraordinary burden: conservator, IHSS provider, primary caregiver for a medically catastrophic 21-year-old AND a 78-year-old with dementia. Father works 10-12 hrs/day. Sister Isabella is 17 and helps but has her own life. Caregiver collapse is a critical and imminent risk. Priorities: (a) maximize IHSS hours; (b)

Regional Center respite services; (c) explore adult day health for Espie (medically complex — limited options); (d) explore adult day program or in-home attendant for grandmother; (e) connect Maria with caregiver support group (ideally Filipino community or medically complex child/adult caregiver group); (f) assess Maria's physical and mental health every visit; (g) cultural competence — respect Maria's faith-based coping and Filipino cultural practices.

PLAN OF CARE GOALS (60-DAY CERTIFICATION PERIOD)

#	Goal	Target
1	Zero milrinone interruptions or infusion-related adverse events	08/09/2025
2	Zero CLABSI or Hickman complications	08/09/2025
3	Zero peritonitis episodes; PD adequate (weekly Kt/V ≥ 1.7)	08/09/2025
4	Albumin improved to ≥ 2.3 (from 2.0)	08/09/2025
5	Weight stable or gaining (target ≥ 80 lbs)	08/09/2025
6	Zero plastic bronchitis episodes requiring hospitalization	08/09/2025
7	SpO2 maintained $\geq 88\%$ for $\geq 95\%$ of monitored time	08/09/2025
8	INR within target (2.5-3.5) for $\geq 70\%$ of draws	08/09/2025
9	Zero seizure events	08/09/2025
10	Sacral wound improved (Stage 1 or resolved)	08/09/2025
11	Right elbow eschar stable (no deterioration)	08/09/2025
12	SIB episodes reduced by $\geq 50\%$ from baseline (behavioral tracking)	08/09/2025
13	Patient uses AAC device for ≥ 10 functional communication exchanges per day	08/09/2025
14	All caregivers (mother, father, sister) demonstrate competence in: milrinone pump, Hickman care, PD setup/disconnect, J-tube management, CoughAssist, BiPAP, seizure protocol, emergency medications	07/15/2025
15	IHSS hours increased to maximum authorized	07/31/2025
16	Respite services activated for mother	07/31/2025
17	Transplant evaluation progressing; all required pre-transplant workup completed	08/09/2025
18	Zero ED visits, zero unplanned hospitalizations	08/09/2025
19	Grandmother Consuelo's care needs assessed and addressed (separate plan)	07/31/2025
20	Monthly multi-agency care conference established	07/01/2025

PATIENT / CONSERVATOR SIGNATURES — EVALUATION 7

Conservator Signature: _____ Date: _____

Print Name: Maria Santos Reyes

Relationship: Mother / Limited Conservator (San Mateo County Superior Court Case # _____)

Legal Authority: Letters of Conservatorship — copy on file

(Patient unable to sign — non-verbal, severe intellectual disability; conservator signature constitutes legal consent for all services)

Interpreter Used: ☐ Yes — Tagalog telephone interpreter for all complex medical discussion

Clinician Signature: _____ Date: _____

Print Name / Title / Credentials: _____

Patient / Representative / Conservator

Mock signature area — training only.

Evaluating Clinician

Mock signature area — training only.

Caregiver / Interpreter if applicable

Mock signature area — training only.

Date / Attestation

Mock signature area — training only.